

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°139/MVE-BDEV/30/09/2026



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HIGHLIGHTS (last 24 hours)

- As of September 30, 2026, there were 76 new confirmed cases of Ebola Virus Disease (EVD), including 28 deaths. Community transmission cases were reported. These cases originated from the provinces of Ituri (43 cases), North Kivu (31 cases), and Haut-Uélé (2 cases). In addition, 8 deaths were recorded in Ebola Treatment Centers (ETCs).
- A total of 474 new samples were analyzed; 76 of these samples were found to be positive for the Bundibugyo virus, representing a positivity rate of 16.0%.
- No new health zones have reported any confirmed cases in the last 24 hours.

ACCUMULATIVE CAS 8 300	ACCUMULATIVE DEATH CONFIRMED 4 018	LETHALITY 48,4 %	PATIENTS IN ISOLATION/CTE 842	ACCUMULATIVE HEALED 2 162	TRACKING RATES CONTACTS (Of the Day) 80,6 %
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Affected provinces and affected health zones



7 Provinces affected

Uele, Ituri, North Kivu,
South Kivu, Tshopo, Lower Uele and
Sud Ubangi



63 out of 167
health zones affected
(37,7%)

Summary of key points from the last 24 hours

In the past 24 hours, 76 new confirmed cases of Ebola Virus Disease (EVD), including 28 community deaths, have been recorded in 3 provinces. The provinces reporting these cases are Ituri (43 cases), North Kivu (31 cases), and Haut-Uélé (2 cases).

In addition, 8 deaths were recorded in the CTEs (6 in Ituri and 2 in North Kivu), bringing the total number of confirmed deaths for the period under consideration to 36.

However, 41 patients (27 in Ituri, 7 in North Kivu, 5 in Haut-Uélé and 2 in Tshopo) were declared cured after obtaining two negative results for Ebola virus disease.

As of September 30, 2026, the DRC had recorded 8,300 confirmed cases and 4,018 deaths.

(Case fatality rate: 48.4 %). Ituri remains the epicenter of the epidemic with 75.8% of cumulative confirmed cases reported and 56.6% of new confirmed cases. In contrast, North Kivu province has a case fatality rate higher than the national average, at 59.3%.

In total, 63 out of 167 health zones (37.7%), spread across seven provinces, have been affected since the start of the epidemic.

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 30, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	43	6293	2 902	46,1%	28/36 (77,8 %)
North Kivu	31	1601	950	59,3%	16/34 (47,1 %)
Haut-Uélé	2	346	144	41,6%	7/13 (53,8 %)
Boss	0	45	16	35,6%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	10	4	40,0%	3/11 (27,3%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Total	76	8 300	4 018	48,4%	63/167 (37,7 %)

Table 2. Distribution of confirmed cases and deaths by province and zone as of September 30, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	6293	2902	46,1%	43	11	6	17
Give	11	1	9,1%				0
Underland	8	6	75,0%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	202	46	22,8%				0
have	2	2	100,0%				0
Bunia	1739	571	32,8%	11	3	4	7
Ladies	40	9	22,5%				0
Drodoro	15	7	46,7%				0
Fireworks	82	31	37,8%				0
Gethy	7	3	42,9%				0
Flounder	2	0	0,0%				0
Kilo	37	12	32,4%	2	0	0	0
Team	169	103	60,9%	0	0	1	1
Liter	287	145	50,5%	3	2		2
Logo	13	5	38,5%				0
Fight	34	9	26,5%				0
distributed	4	1	25,0%				0
Washing	32	11	34,4%	3	0	0	0
Darkness	138	75	54,3%	1	0	0	0
Mangala	357	154	43,1%	6	0	0	0
Mongbwalu	703	333	47,4%	2	2	0	2
Nanny	305	167	54,8%	7	4	1	5
Nizi	781	279	35,7%	3	0	0	0
Legumes	129	37	28,7%				0
Jungle	12	4	33,3%				0

Rwampara	1088	366	33,6%	5	0		0
Tchomia	76	25	32,9%				0
A valves	THAT	493	THAT				0
North Kivu	1601	950	59,3%	31	17	2	19
Me	361	239	66,2%	13	9	1	10
Farm	19	7	36,8%				0
Butembo	267	171	64,0%	4	1	0	1
Ten	1	0	0,0%				0
Kalunguta	56	19	33,9%	2	1	0	1
Cut	566	359	63,4%	4	2	0	2
Source	1	0	0,0%				0
Kyondo	33	20	60,6%				0
Lubero	21	11	52,4%				0
Be careful.	36	14	38,9%	3	2	0	2
Manguredjipa	13	5	38,5%			0	0
Masereka	29	10	34,5%	0	0	1	1
The museums	126	68	54,0%	2	1	0	1
Mutwanga	14	2	14,3%	2	0	0	0
Oicha	27	15	55,6%	1	1	0	1
Vuh	31	10	32,3%				0
Haut-Uélé	346	144	41,6%	2	0	0	0
Kil Mangbetu	39	17	43,6%				0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	102	44	43,1%				0
Lining	87	39	44,8%	2	0	0	0
Beans	2	1	50,0%				0
Ordinary	112	41	36,6%				0
Boss	45	16	35,6%	0	0	0	0
Bafwasende	7	1	14,3%				0
Kabondo	5	1	20,0%				0
Lubunga	1	0	0,0%				0
Maquis-Kisangani	24	10	41,7%				0
Mangobo	6	3	50,0%				0
Boss	1	0	0,0%				0
Wanie-Rukula	1	1	100,0%				0
South Kivu	3	1	33,3%	0	0	0	0
Mili-Murhesa	3	1	33,3%				0
Bas Uélé	10	4	40,0%	0	0	0	0
Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Walk	3	2	66,7%				0
Sud Ubangi	2	1	50,0%	0	0	0	0
Hair	2	1	50,0%				0
Total	8300	4018	48,4%	76	28	8	36

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified by province for September 30, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	29	6	35	15	6	14	0	21	14
Bas Uélé	133	5	138	13	5	120	0	18	0
Ituri	987	61	1 048	131	61	49	0	175	106
North Kivu	1 232	49	1 281	136	49	775	0	185	93
Sud Ubangi	53	0	53	1	0	48	0	0	0
Boss	132	1	133	7	1	125	0	8	0
Total General	2 566	122	2 688	303	122	1 131	0	407	213

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

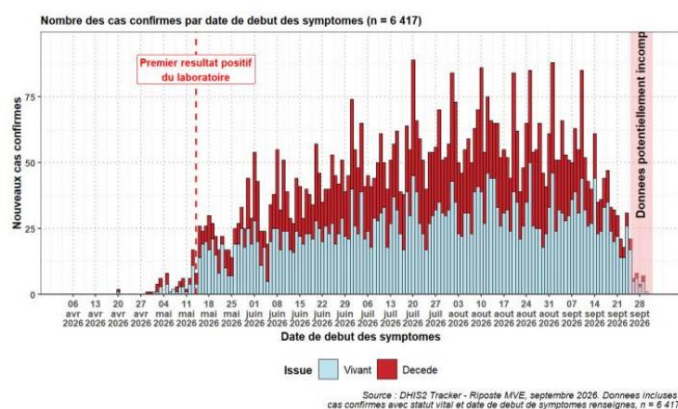
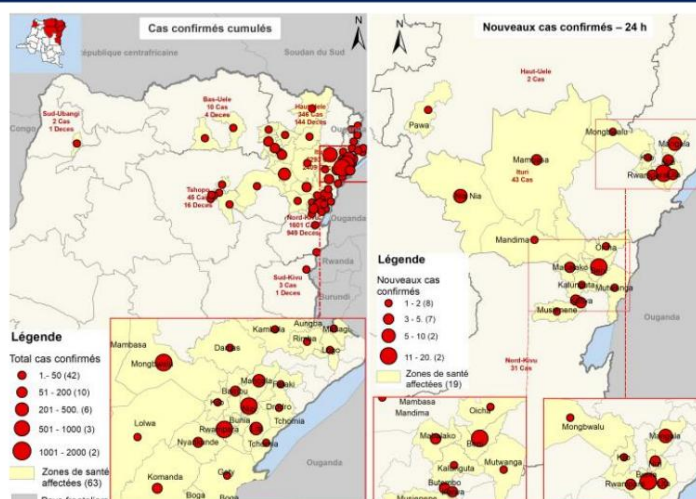


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

By the end of the day on September 30, 2026, **2,688 alerts** had been received, **1,556** (57.9%) of which were verified, including **425** (27.3%) validated as suspected cases. Of these, **407** (95.8%) have been investigated. Thus, 213 suspected cases were transferred to CT/CTE (Table 3).

Of the **32,363** contacts currently being monitored, **26,089** were seen in the last 24 hours, representing a follow-up rate of **80.6 %**, slightly higher than the previous day (78.7%) but still below the required threshold (95.0%). This rate was 93.9% (92/98) in South Ubangi, 82.1% (11,965/14,577) in Ituri, 87.3% (660/756) in Tshopo, 87.0% (161/185) in Bas-Uélé, 79.1% in North Kivu (11,595/14,665) and 77.6% in Haut-Uélé (1,616/2,082).

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

Six (6) alerts were notified to the PoE/PoCs during the day, four (4) in Ituri and two (2) in North Kivu. In Ituri, at the Mabakese PoC (Mambasa Health Zone), the alert concerning a 19-year-old traveler from Beni was validated, and the suspect was then transferred to the Mambasa CTE (transfer rate of 100%). A second alert at the same PoC and the alert at the Sans Complainte PoC (Ariwara Health Zone) were invalidated after secondary screening. At the Foner Mambasa PoC, the body of a person arriving from Mandima General Hospital was intercepted and swabbed; results are pending. In North Kivu, the two alerts from the Mukulia and Kasitu PoCs were invalidated after investigation.

Table 4. Status of key indicators for PoE/PoC by province, as of September 30, 2026

Indicators	Ituri	North Kivu	Boss	Global
Report completeness	96,7% (58/60)	95,2% (20/21)	100,0% (18/18)	97,0% (96/99)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	143 836	100 400	20 058	264 294
% of travelers screened	97,7%	97,5%	98,9%	97,7%
% of travelers who washed their hands	97,4%	97,5%	98,4%	97,5%
% of travelers made aware	99,5%	97,5%	100,0%	98,8%
Number of alerts notified	4	2	0	6
Number of validated live alerts (suspected cases)	1	0	0	1
Number of bodies intercepted today % of	1	0	0	1
suspected cases transferred to CTE/CT	100,0%	THAT	THAT	100,0%
% of lifeless bodies swabbed/secured	100,0%	THAT	THAT	100,0%
% of suspected cases investigated within 4 h	100,0%	THAT	THAT	100,0%

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 43 new positive results** (32 living and 11 deaths) out of 209 samples received and analyzed (**positivity: 20.6%**).
- **North Kivu: 31 new positive results** (14 living and 17 dead) out of 230 samples received and analyzed (**positivity: 13.5%**).
- **Haut-Uélé: 2 new positive results** (2 living and 0 deaths) out of 18 samples received and analyzed (**positivity: 11.1%**).
- **Tshopo: 10 samples received and analyzed** (10 live), all came back negative.
- **Bas Uélé: 7 samples received and analyzed** (5 living and 2 dead), all came back negative.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 23 out of 41 rings were opened (56.1%), 78 out of 80 households were decontaminated (97.5%), and 8 out of 8 health facilities were decontaminated (100%). Of 76 EDS alerts (including 5 postponements), 54 EDS were carried out (71.1%) and 55 bodies were swabbed; 30 schools in Mongbwalu received IPC kits.
- **North Kivu:** 15 out of 21 rings were opened (71.4%), 5 out of 27 households were decontaminated (18.5%), and 28 out of 30 health establishments were decontaminated (93.3%). In total, 198 health facilities were monitored, 628 service providers were briefed, and 33 blood and body surveys were conducted, with 43 bodies swabbed out of 45 alerts received (95.6%).

- **Haut-Uélé** : 2 out of 2 rings were opened (100%), 9 out of 9 households were decontaminated (100%), and 2 out of 2 ESSs were decontaminated (100%). Out of 6 EDS alerts, 5 EDSs were carried out (83.3%), and 5 bodies were swabbed in Isiro and Wamba; UNICEF provided the Isiro health zone with 2 vehicles and a decontamination kit.
- **Tshopo** : 17 health establishments (HEs) were monitored in 6 health zones and 179 service providers were briefed; integrated training of 150 private HEs continues in Makiso-Kisangani, and the household of case 45, the Vita Nova clinic (case 44), and the Mangobo General Referral Hospital have been decontaminated. Cumulatively, 36 out of 36 HEs (100%) and 27 out of 31 households (87.1%) that hosted a confirmed case were decontaminated within 48 hours.
- **Bas-Uélé** : 9 out of 10 planned ESS (90.0%) were monitored in Viadana, Buta and Ganga and 38 Contractors briefed (100%); 3 out of 4 sorting facilities are operational in Buta and work on the Buta CTE is continuing. Five bodies were swabbed (3 in Ganga, 1 in Buta and 1 in Viadana), without a DSS (Data and Safety Survey) carried out.

2.4.2. Challenges

In **Ituri**, the opening of the rings remains insufficient (56.1%; 23/41), other bodies have not been swabbed due to a lack of supplies and teams in Mandima or following refusal by families, resistance persists in Mangala and Fataki, and no households or health establishments (ESS) that were decontaminated have received kits. In **Tshopo**, the shortage of infection prevention and control (IPC) supplies in health establishments and the breakdown of the incinerator at the Makiso-Kisangani General Referral Hospital (HGR) persist. In **Bas-Uélé**, the Bomea and Triangle health centers have neither a functional waste area nor hygienic latrines. In **Haut-Uélé**, none of the 9 households or 2 health establishments that were decontaminated have received kits.

2.5. Holistic care

2.5.1. Main actions

- In **Ituri**, 90 new admissions were recorded compared to 80 discharges, including 27 recoveries. A total of 398 patients remained in isolation at the end of the day. The bed occupancy rate was **39.2%** ; 360 patients received hot meals, and children receiving LNPE/SLM were fully covered at the CTE (13/13), in daycare (5/5), and in the community (50/50); 38 patients are in critical condition.
- In **North Kivu**, 76 new admissions were recorded, compared to 71 discharges (7 recoveries). A total of 360 patients remained hospitalized, including 256 in standardized facilities. The bed occupancy rate was **72.3%** ; 20 patients were in critical condition at the Beni Ebola Treatment Center, and 127 patients received hot meals.
- In **Tshopo**, no new admissions were recorded, while 3 discharges were reported. Four (4) patients remained in isolation at the end of the day, representing an occupancy rate of **12.9%**.
- In **Bas-Uélé**, 10 new admissions were recorded compared to 4 discharges at the end of the day. Eleven (11) patients remained in isolation at the end of the day.
- In **Haut-Uélé**, 11 new admissions were recorded against 18 discharges, including 5 recoveries.
In total, 69 patients are in isolation in the CTEs, representing an occupancy rate of **50.0%** ; 71 patients, 100 accompanying persons and 40 PPLs received hot meals.

2.5.2. Challenges

- In **Ituri**, insufficient capacity of some CTE/CT facilities and inadequate care food-related issues remain a concern.
- In **Bas-Uélé**, the lack of a standardized CTE and ambulance for the transfer of patients remains a major operational challenge.
- In **Tshopo**, the continued presence of confirmed cases in isolation necessitates the maintenance of infection prevention and control measures.

2.6. Vaccination

2.6.1. Main actions

- **In Ituri**, 86 people were vaccinated (29 women and 57 men, including 1 pregnant woman and 2 breastfeeding women), with no reported adverse events following immunization (AEFI), bringing the total number of people vaccinated to 817 (348 women and 469 men). A temporary stock shortage of ERVEBO led to the temporary suspension of vaccination and research activities.
- **In Bas-Uélé**, vaccination of PPL and TPL continued in Buta, bringing the provincial total to 1,034 people vaccinated out of a target of 13,202 (7.8%), primarily in Buta (584; 20.4%) and Ganga (273; 20.5%). Two compassionate vaccination teams were deployed to support the health zones of Aketi and Viadana.

2.6.2. Challenges

In Ituri, the target for PPL/TPL remains to be redefined, which prevents the calculation of a reliable coverage rate.

In Bas-Uélé, the UCCs have not been put into operation, the validated micro-plan and vaccination funds are not available.

2.7. Continuity of Care

2.7.1. Main actions

- **In Ituri**, the visit to the Bunia General Referral Hospital (HGR) made it possible to prioritize the activities of its recovery plan: the physical separation of the HGR/CTE and the 24-hour triage are effective, but the emergency service is not functional and attendance is declining (213 new cases in July 2026 compared to 747 in July 2025).
- **In Haut-Uélé**, the evaluation of the Nebobongo Health Center (Pawa Health Zone) reveals a drop in attendance of more than 40% and only 43% of functional services (major disruption); the Pawa Health Center was also evaluated and 2 PPLs are being monitored at the Wamba CTE.
- **In Tshopo**, the Horizon Medical Centre was evaluated (2 out of 11 staff vaccinated, i.e. 18.2%, and triage area not effective), the CM CELPA recovery plan was developed and the training of zonal focal points on continuity of services began.
- **In Bas-Uélé**, the evaluation of the Ganga/Dingila health zone on vaccination (May to August 2026) shows a Penta 1 coverage between 84.5% and 101.7% and a Penta 2 coverage between 80.1% and 93.3%, compared to an expected 95%, with a dropout rate of 5.2% to 8.9%.

2.7.2. Challenges

In Tshopo, there has been a decrease in the use of services, particularly maternity and cesarean sections, in the assessed ESS; the temporary closure of the DHKis ESS remains proposed in the face of the multiplication of confirmed and suspected cases.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri**, 6,900 home visits reached 26,866 people; 724 educational talks brought together 8,108 people, 6 community debates reached 325 people, and 393 mass awareness campaigns reached 5,866 people; and 81 public service announcements were broadcast on 36 radio stations. A total of 170 community alerts, including 85 fatalities, were reported; 83 out of 86 (96.5%) received feedback, and the 5 recorded incidents were resolved within 48 hours (8 out of 28 health zones completed).
- **In North Kivu**, 12,509 home visits reached 36,982 people, 133 educational talks reached 2,936 people, and 41 mass communication sessions reached 5,424 people; 277 out of 650 expected community alerts (42.6%) were notified, and 20 out of 25 feedback requests (80.0%) were resolved within 48 hours. CREC facilitated the community reintegration of four recovered individuals in Vuhovi and Biena, as well as the training of 40 community leaders.
- **In Tshopo**, 287 home visits reached 1,606 people, 6 educational talks reached 251 people, and 8 mass awareness sessions reached 3,018 people; 27 community alerts were notified (18 in Makiso-Kisangani and 9 in Tshopo) and the feedback received was resolved (1/1), while awareness-raising activities in schools continued in Mangobo and Makiso
- **In Bas-Uélé**, 102 home visits reached 1,184 people, 6 educational talks reached 43 people, and 3 mass awareness campaigns reached 570 people in Viadana, Titule, and Ganga; the 12

community alerts and the 3 notified feedbacks have been addressed, and 31 community and religious leaders have been sensitized in the Limba AS (Viadana).

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **In North Kivu**, 16 out of 17 confirmed cases (94.1%) recorded the previous day and 246 confirmed cases under follow-up benefited from SMSPS interventions in 6 out of 7 health zones that reported (85.7%). The teams took charge of 112 new suspected cases and 406 suspected cases under follow-up, assisted 24 out of 34 households with separated and orphaned children (70.6%), announced 98 results, including 12 positive ones, and reintegrated 7 recovered individuals.
- **In Ituri**, 19 out of 32 confirmed cases (59.4%) from the previous day, 87 confirmed cases under observation out of 126 (69.0%), 29 new suspected cases out of 83 (34.9%), and 109 suspected cases under observation out of 134 (81.3%) benefited from SMSPS interventions. The teams supported 262 caregivers and 49 people living with HIV, monitored 30 out of 34 recoveries in the community (88.2%), and announced 67 results, including 19 positive ones.
- **In Tshopo**, the teams provided psychological support to 2 confirmed cases at the CTE Météo, carried out psychoeducation for 7 suspects and 123 people, including 92 students in Mangobo, and announced 2 negative results; 2 recovered individuals were followed up in the community (100%).
- **In Haut-Uélé**, 1 new confirmed case, 20 confirmed cases under follow-up, 7 new suspected cases and 5 suspected cases under follow-up benefited from SMSPS interventions in Isiro, Boma Mangbetu and Wamba; the teams supported 56 accompanying persons and 66 PPLs, announced 7 results, including 3 positive ones, and accompanied the release of 4 recovered patients (data from Pawa not transmitted).

2.9.2. Challenges

- **In Ituri**, 13 out of 28 designated health zones are covered (46.4%); psychologists and agents psychosocial factors identified in Mandima must be urgently reinforced for interventions around cases.
- **In Haut-Uélé**, difficulties in the mobility of APS in Wamba and Boma Mangbetu, insufficient nutritional kits for weaned infants and poor data reporting in Pawa and Boma Mangbetu.
- **In Tshopo**, there is a need to train the APS (Auxiliary Security Personnel) of the ZS (Health Zones) of Wanie-Rukula and Bafwasende, and insufficient means of communication and movement of the pillar.

2.10. Logistics

2.10.1. Main actions

- **In Tshopo**, followed by the extension of the CTE Météo to a capacity of 30 beds (addition of 8 rooms) and the construction of the CTE in Bafwasende, finalization of a fuel supply agreement in Bafwasende for the ambulance (WHO) and support for the mobility of the pillars.
- **In Bas-Uélé**, deployment of surveillance teams in Ganga and Viadana, continuation of the rehabilitation of the meeting room and offices of the DPS (WHO) and of the construction of the transit center of the HGR of Buta; the stock of chlorine is limited to 25 kg and diesel is out of stock.

2.11. Security

2.11.1. Main actions

- **In North Kivu**, the Risk Mitigation and Security pillar carried out 6 security coverage activities (100%), including support for EDS teams for the burial of 2 confirmed cases at the Bundji and Masiani cemeteries and for 3 swabs in the Katwa health zone, as well as for the IPC team for the decontamination of 3 rooms at CM Rwenzori (AS Ngaliika).

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

- In **Ituri**, 758 people (552 women and 206 men) were sensitized on the PSEA in the community in Bunia, Aru, Rimba and Mongbwalu with the support of WHO and Solidarité; only 13 out of 36 health zones (36.1%) know the reporting mechanism.
- In **Haut-Uélé**, 78 people, including 25 service providers who all signed the code of good conduct, were sensitized on the PSEA and the reporting mechanism (495555/151) in the Mayogo AS (Isiro Health Zone), including pregnant women received in CPN at the VDO Health Center.

2.12.2. Challenges

At the national level, only 2 DPS reported their PSEA activities (28.6%) and CBCMs remain largely ineffective.

MESSAGES CLES

- Active and sustained community transmission remains observed in the provinces of Ituri, North Kivu, and Haut-Uélé. Meanwhile, the province of Sud-Ubangi has not reported any confirmed cases in the last 24 hours, and a lull has been observed in the province of Tshopo compared to the last 72 hours.
- Progress to be consolidated has been made in the field of epidemiological investigations: Almost all **(95.8 %)** of the cases suspects were investigated within 24 hours of being notified. However, the contact tracing rate is higher than the previous day **(80.6% vs. 78.7%)**. Nevertheless, efforts must be made to bring this rate up to the required threshold (95%).
- Strengthen surveillance, care, IPC, logistics and community engagement, prioritizing increased capacity of CTE/CT in North Kivu, rapid transfer of suspects and isolation of confirmed cases.
- Strengthen preparedness and response to Ebola Virus Disease (EVD) in the river corridor and surrounding areas, particularly in South Ubangi, through active case finding, contact tracing, and ensuring the availability of supplies. Continue cross-border surveillance with South Sudan.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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